

|             |                    |            |                                  |           |                    |
|-------------|--------------------|------------|----------------------------------|-----------|--------------------|
| Name        | : Saroj Agarwal    | Age/Gender | : 63 y / F                       | Collected | : 26/03/2026 01:57 |
| Patient ID  | : 0010663186       | Visit Type | : IP                             | Received  | : 26/03/2026 03:20 |
| Referred By | : Dr. Darshit Shah | DOB        | : 27/08/1962                     | Reported  | : 31/03/2026 10:15 |
| Accession   | : 2600066738       | Location   | : 1T13S`1TR1322`1TB1322`1ONCOMED |           |                    |

**DEPARTMENT OF LABORATORY MEDICINE - MICROBIOLOGY**

**Test Name**

**Result**

**Blood Culture Set**



**Culture Automated Blood Aerobic**

Report Status

Final Report

Specimen

Blood

Site

Right femoral sheath

Culture

No Growth after 5 days of Incubation

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**MD Microbiology  
Consultant Microbiologist**

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## DEPARTMENT OF LABORATORY MEDICINE - MICROBIOLOGY

### Test Name Result

**Method/ Principle of BacTAlert Virtuo analyser:** BacT/ALERT Virtuo Microbial Detection Systems utilize a colorimetric sensor and reflected light to monitor the presence and production of carbon dioxide (CO<sub>2</sub>) that is dissolved in the culture medium. If microorganisms are present in the test sample, carbon dioxide is produced as the organisms metabolize the substrates in the culture medium. When growth of the microorganisms produces CO<sub>2</sub>, the color of the gas-permeable sensor installed in the bottom of each culture bottle changes to yellow. Neutralizes all classes of antibiotics by FAN Plus method that normally hinder growth in manual blood culture methods.

Positive flagged cultures are processed on culture media. Identification by automated MALDITOF/Vitek 2 and or conventional methods, sensitivity on vitek 2. Certain antibiotics tested by Kirby Bauer's method and MIC is not reported for antibiotics which are deduced by software as per CLSI. Antisera tested as and when applicable.

**Remark:**

1. Results of culture and antimicrobial susceptibility test needs to be correlate clinically.
2. Previous history of antibiotic usage may influence the growth of microorganisms in vitro.

| Colony Count                                                                                                                   | Interpretation                                                                                                                                                            |
|--------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Colony Counts of 10000 to >= 100000 CFU/ml of single/two Potential pathogen/s.                                                 | Significant uropathogen. Suggestive of Urinary tract infection (UTI) needs treatment based on antimicrobial susceptibility test results.                                  |
| Colony counts between 1000 to 10000 CFU/ml of single Potential pathogen.                                                       | Can be considered Significant growth, correlation with Microscopy and Clinical history required.                                                                          |
| Colony counts between 100 to 1000 CFU/ml.                                                                                      | Insignificant growth. Probable commensal contamination during voiding.                                                                                                    |
| Either two or three organism types with predominant growth of one organism type and <10000CFU/mL of the other organism type(s) | Suggestive of Urinary tract infection (UTI) needs treatment based on antimicrobial susceptibility test results. Correlation with Microscopy and clinical history required |
| Any number / Any count.                                                                                                        | Significant in case of Suprapubic aspirates/surgically obtained (e.g. cystoscopy) urine specimens.                                                                        |
| >= 3 organism types with no predominant (10000 >= 100000 CFU/ml) pathogen.                                                     | Fresh specimen required as possibility of contamination during voiding.                                                                                                   |

∞ End of Report ∞

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